

Cash Pay, end to end

How a patient with no insurance moves from intake to a shipped order — what to press, what each answer means, and what to do when one of them does not work.

Written	22 September 2026, revised 23 September
Verified against	the live monday boards and the Railway services on both dates
Audience	Brandon (monday access) and whoever is helping him — including an AI assistant
Source of truth	CLAUDE.md \$5.48 in the command-center-test repo, plus scripts/cash-pay/

How to use this document

Every board id, column id and label id in here was read back from the live boards on the date above, so they can be used directly. Where something is not built, it says so plainly rather than describing the intention as if it were working — section 13 is the honest list of what is still open. If you are an AI assistant reading this on Brandon's behalf: treat the **Status** table on page 2 as the state of the world on the date above, and check anything load-bearing against the live boards rather than assuming it has stayed that way.

1 What a Cash Pay patient is

A Cash Pay patient has no insurance. Nobody is billed, no eligibility check is run, there is no medical necessity to document and no prior authorisation to chase. They pay us directly, out of pocket, for everything in the order.

That breaks the normal pipeline in one specific place: the Intake screen will not let a patient advance until insurance is on file, so a cash pay patient gets stuck at the very first stage. The work described in this document exists to give them a route through.

The case that forced it was **Debbie Hinze** — 90 days of t:slim cartridges, AutoSoft XC 9mm 43" infusion sets and a Dexcom G7, quoted at **\$1,030.69**, stuck at Intake because Advance could not be satisfied. That figure is now the anchor the pricing rule is tested against.

The one rule that shapes everything else

A Cash Pay patient **never touches Medical Evaluation or Insurance**. That is Corey's handoff of 14 August 2026, reaffirmed by Josh on 22 September. There is no medical necessity to gather and no auth to chase, so routing them through those boards — even briefly, even automatically — is the wrong answer however much configuration it would save.

2 Status — is it up and running?

Yes — a cash pay patient can be taken from intake to a paid order today. Five of the six stages are live and each was verified against the real boards and the real services on 22 September 2026 rather than reasoned about: a throwaway patient was advanced, a throwaway order was priced, a real Stripe link was minted and a real text was sent. The test rows were deleted afterwards. **The sixth — the ordering gate — is built but not yet switched on**, so nothing in the software physically stops an unpaid cash pay order going to Cardinal. That is the one thing to know before using this.

Stage	State	What that means in practice
1. Intake marks the patient Cash Pay	LIVE	Choosing Cash Pay hides the insurance sections and the benefits check, and drops every row a patient with no insurance cannot satisfy from the readiness checklist, so Advance works.
2. Skip straight to Welcome Call	LIVE	Advance writes Advance to Welcome Call ; monday automation 7923595946 creates the Welcome Call item and moves this one to Completed. Medical Evaluation and Insurance are skipped entirely.
3. Welcome Call	LIVE	Nothing cash-pay-specific beyond the out-of-pocket card saying there is no benefit to estimate and linking to the Cardinal costs.
4. The order is priced	LIVE	The Cash Pay card renders the full quote — per-line prices and the total.
5. Payment link generated and texted	LIVE	Generate link mints a Stripe payment link for the quoted amount; Send to patient texts it and stamps the date. Both are monday webhooks onto the payment service.
6. Ordering gate	BUILT, OFF	Does not bite yet. The rule that refuses an unpaid cash pay order lives inside the app's own "place this order" button, and that button is still switched off — orders are placed by flipping Order Status on the board, which nothing checks. The manager release <i>is</i> live. See section 10.

What was actually tested, in case you need to repeat it

The skip: a throwaway Profile Clean-Up item marked Cash Pay was advanced — the source landed in Completed and a Welcome Call item appeared carrying Cash Pay, DOB, phone, doctor and serving. **The link:** a throwaway order at \$1.00 was flipped to *Generate link*, which produced a real pay.medicallymodern.com link and cleared the trigger, then to *Send to patient*, which texted it (RingCentral message 3313052761012), stamped Cash Pay Link Sent and cleared the trigger again.

Where the code lives

The app half is on the **command-center-test** repository. Production is a mirror of it, force-pushed only when Josh presses the *Sync from Test Repo* workflow — so unless he has synced since 22 September, the cash pay screens are on test and not yet on the live site. The payment service half is on **coins-form-payment** and is deployed.

3 The path, end to end

How it is meant to run once everything is switched on:

#	Where	What happens
1	Profile Send Off — Intake	A rep sets General Insurance = Cash Pay . The app mirrors that into Primary Insurance , which is the value that travels between boards. Insurance sections and the benefits check disappear; the doctor step stays required.
2	Profile Send Off — Profile Clean-Up	The rep presses Advance to Welcome Call (a different button from the usual Advance to MN). A board automation creates the patient's Welcome Call item and moves this one to Completed. Medical Evaluation and Insurance are skipped entirely.
3	Welcome Call	The usual call: confirm the order, the products, the quantities, the address. The out-of-pocket card says there is no benefit to estimate and points at the Cardinal costs page.
4	New Order Board	The order is created as normal. The Cash Pay card shows the priced quote — each line at Cardinal cost × 1.25, plus a \$10 shipping line on very small orders.
5	New Order Board	The rep presses Generate link , then Send to patient . The first mints a Stripe payment link for the exact quoted amount; the second texts it to them.
6	Stripe → New Order Board	The patient pays. Stripe's webhook writes the charge id, the paid date, and flips Order Status to Paid Cash .
7	New Order Board	Only now can the order be placed with Cardinal. From here it is an ordinary order — the Cardinal poller drives it through shipped and delivered like any other.

Today, stages 2 and 5 do not happen on their own

Until they are switched on, a cash pay patient advances on **Advance to MN** and goes through Medical Evaluation and Insurance like an insured patient, and the payment link is not generated by the app. Section 11 has the practical process for a patient in front of you right now.

4 The marker: how a board knows it is a cash pay patient

One value carries the whole thing: a **Cash Pay** label on the insurance column. A rep sets it once at intake on General Insurance; the app mirrors it into Primary Insurance, and it is Primary Insurance that rides the board-to-board hops.

Label ids are different on every board, and that is not a mistake

monday assigns a label's id from its colour when the label is created, so the same word has a different number on each column. Four of these happen to be 152, which is luck rather than a rule — never assume a fifth board's id. Read it back from the column. A write to an id a column does not have is accepted silently and does nothing.

Board	Column	Column id	Cash Pay label id
Profile Send Off	General Insurance	color_mm24ap4j	16
Profile Send Off	Primary Insurance	color_mm1xg10n	152
Welcome Call	Primary Insurance	color_mm1x157j	152
Subscription	Primary Insurance	color_mm254qxj	152
New Order Board	Primary Insurance	color_mm18jhq5	152

There is deliberately no Cash Pay label on Medical Evaluation or Insurance

Those two boards are never meant to see a cash pay patient, so a label there would only be reachable by the route this whole design exists to avoid. If you find yourself wanting one, the design has gone wrong somewhere upstream.

5 Stage 1 — Intake

Live. On either intake route — Referral Intake or the DTC intake page — picking **General Insurance = Cash Pay** changes the screen:

- The **Verified Insurance** section is not rendered.
- The **Run benefits check** and **Start Insurance Follow-Up** buttons are replaced by a short note saying why, and where the price comes from instead.
- On the Referral Intake route, **Run Stedi Check** goes too — there is no payer for Stedi to ask about, so a run could only ever come back as an eligibility error that reads like a data problem.
- The insurance rows leave the readiness checklist, so **Advance** can be satisfied. This is the bit that unblocks the patient.
- The remaining steps renumber, so the rep is not looking at a list that starts at 2.

The doctor step is still required, and that is deliberate

Cardinal's order payload carries a mandatory doctor block, so hiding the doctor step would let cash pay orders reach Cardinal with an empty doctor and fail hours later — long after the only moment when somebody was on the phone with the patient. Josh's call, 21 September.

Each hidden control is replaced by a sentence explaining itself. A card that simply stops where a rep expects a button reads as a broken page, and that is how people learn to distrust a screen.

6 Stage 2 — the skip to Welcome Call

Live. Automation **7923595946** on Profile Send Off does the whole hop.

Trigger	Move to Onboarding color_mm1zmeb3 changes to Advance to Welcome Call (label id 6)
Then	Create an item on Welcome Call (18410804557), group Welcome Call group_mm1wvq8p, named after the trigger item, carrying the patient's columns
Then	Move the Profile Send Off item to Completed group_mm1y57sz

Board automation **7917676280** is untouched. It triggers on *Advance to MN* specifically, so an insured patient's route is byte-identical to what it has always been.

Six columns are deliberately not carried

Both **Coverage Paths**, **Stedi Home Plan**, **Stedi Coinsurance %**, **Stedi Plan Begin Date** and **Referral?** arrive blank on the Welcome Call item, and that is the decision rather than a gap — a coverage path is how a payer covers a product, and a cash pay patient has no payer. **Profile Send-Off Notes** IS carried, which is the one that mattered: it is the intake case history the Welcome Call rep reads before the call.

23 mappings still point at Medical Evaluation columns — harmless, but worth tidying

The automation was duplicated from the *Advance to MN* hop, and changing the destination board does not repoint the column mappings. They write nothing. The reason to delete them anyway is that the risk they carried was a silent one: a create-item step holding column ids the destination board lacks *could* have been refused outright, creating no Welcome Call item while the source item moved to Completed — a patient out of the pipeline with nothing erroring. Tested: monday ignores them. `scripts/cash-pay/README.md` lists all 23.

7 Stage 3 — Welcome Call

Live, and nothing cash-pay-specific is required. The call runs as it always does: confirm the products, the quantities, the address.

The one difference is the out-of-pocket card. For an insured patient it estimates what the order will cost them; for a cash pay patient there is no benefit to estimate, so instead of printing an error it says so and links to the in-app Cardinal costs page. It deliberately does **not** quote a figure: the quantities are still being negotiated on the call, so a number here is one the rep would read out and then change. The price becomes real on the order, where Stripe fixes it.

8 Stage 4 — pricing the order

Live. The Cash Pay card on the New Order Board renders the full quote.

The pricing rule

Step	Rule
1	Take each product line's Cost from the Cardinal SKU Tracker — the live scraped cost, not a guess — and multiply by the quantity on the order.
2	Multiply by 1.25 and round to the cent, per line .
3	If the total markup across the order is under \$10 , add a separate \$10 “Shipping & handling” line. 25% alone does not cover posting one box of cartridges.

Worked against Debbie Hinze's order: \$30.95, \$71.94 and \$57.32 of tracker cost across the three lines gives \$824.55; at $\times 1.25$ per line that is $\$116.06 + \$269.78 + \$644.85 = \mathbf{\$1,030.69}$ — the figure she was quoted, to the cent. That order is the regression test; if it ever stops matching, the rule has drifted away from a price a human signed off.

A line we cannot price refuses the whole quote

The tracker carries real rows whose cost reads zero, and a missing row looks identical to a product Cardinal has stopped selling. Rather than quietly leave a product out of the total — which would quote the patient short — the card refuses to show a quote at all and says which line it could not price.

The quote is honoured once sent

Tracker costs move daily, and Stripe fixes the amount the moment the link is created. An order priced on Monday ships at Monday's price. The margin absorbs it; nothing re-prices behind the patient's back.

9 Stage 5 — the payment link

Live. Two presses on the Cash Pay card, deliberately: **Generate link** mints it, **Send to patient** texts it.

How it works

Deliberately the same mechanism the existing coinsurance payment flow uses, which has run for a year: **the board is the trigger**. No browser ever holds a payment token.

#	What happens
1	The app writes the quoted total into Cash Pay Amount <code>numeric_mm7devxs</code> , waits until monday confirms it is stored, and only then sets Cash Pay Action to Generate link .
2	A board automation sees that status change and calls the payment service.
3	The service reads the amount off the row, creates a Stripe Payment Link for it, writes it back into Cash Pay Link <code>text_mm7dzgzd</code> , and clears the action column.
4	The rep presses Send to patient . A second automation texts the link and stamps Cash Pay Link Sent .
5	The patient pays. Stripe's webhook writes the charge id and the paid date, and sets Order Status to Paid Cash .

Cash Pay Action label	Id
Generate link	0
Send to patient	3
Link failed	2

Why the amount is written first and verified

monday returns success on a column write before the value is actually readable. The webhook reads that cell the instant the automation fires, so firing the trigger first would mint a link for the previous amount, or for a blank. Writing and confirming the amount before flipping the trigger is what makes that impossible.

Pressing the button again clears the column first

A status write onto the value a column already holds is accepted by monday, fires nothing, and records nothing. So chasing an unanswered link, or retrying after a failure, would be a green toast and no email. The app clears the action column and re-writes it, which is one webhook, not two.

The known trade-off

Because the board webhook carries only an item id and a status label, the payment service can see one number — the total on the row — and not the three product lines. **The Stripe page therefore shows one line**, “Medically Modern — diabetes supplies”, rather than an itemised list. The amount is identical and the itemisation is on the card the rep is reading from. Rebuilding the lines inside the payment service would mean a second copy of the pricing rule in a second repository, whose drift would be a patient charged an amount no screen ever showed.

This is Josh's to accept or change; an itemised route exists and is tested but is not wired up.

The two webhooks

Webhook	Fires on	Does
641115241	Cash Pay Action → Generate link	Mints the Stripe payment link, writes it to Cash Pay Link, clears the trigger
641125712	Cash Pay Action → Send to patient	Texts the link, stamps Cash Pay Link Sent, clears the trigger

Three things about monday webhooks that cost real time here

monday's automation builder has no “send a webhook” action — only a “when a webhook is received” trigger — so these are API webhooks, like every other integration on that board. **The secret rides in the URL path**, because monday signs every delivery with its own JWT in `Authorization`, and a service that reads the header first never compares the real key and refuses a correct webhook with 401. **monday suspends an endpoint that keeps failing and still lists the webhook**, with no status field to read — after fixing a 401 you must delete and recreate it.

The save-time challenge proves nothing about the secret

It is answered before the auth check, so monday will happily save a URL with a typo'd key and then refuse every real event. Test with a real flip, never with the save.

10 Stage 6 — the ordering gate

Read this before you rely on it

The gate is written and tested, and it is not yet switched on. It sits inside the Command Center's own *Mark as Ordered* button — and that button is behind a separate switch that is still off, because the Command Center does not place orders yet. Today an order is placed the way it always has been: a person flips **Order Status** → **Ordered** on the New Order Board, and nothing checks whether the patient paid. **Until that switch flips, the only thing standing between an unpaid cash pay order and Cardinal is somebody reading the Cash Pay card before they flip it.** The card says in plain words whether the order is paid.

What the gate will do once it is on: an unpaid cash pay order cannot be sent to Cardinal, so goods do not leave before money arrives.

The gate reads the **payment columns** — the Stripe charge id and the paid date — and not the “Paid Cash” status label. That distinction matters: the label predates this work and sits on historical cash orders that are already delivered, so a label-based gate would read a finished order as ready to place. The charge id is written by exactly one thing, the Stripe webhook, so it means what it says.

There is a deliberate way through, and it is manager-only

Patients pay by cheque and over the phone — Janelle on Debbie Hinze: *“she is older and does not have Venmo.”* A Stripe-only gate would leave those orders unplaceable forever. A manager can release the order, but must type a reason, and that reason is stamped into the order's notes. It is the only record anywhere of why goods went out against no Stripe payment, which is why it is a typed reason and not a confirmation dialog.

11 What a rep actually does

#	Do this	Note
1	At intake, set General Insurance = Cash Pay .	The insurance sections disappear and Advance becomes satisfiable. The app mirrors it into Primary Insurance, which is the value that travels.
2	Fill in the doctor. It is still required.	Cardinal's order payload needs one. Skipping it fails the order hours later, long after anyone is on the phone with the patient.
3	Press Advance .	They go straight to Welcome Call. No Medical Evaluation, no Insurance.
4	Run the Welcome Call as normal.	The out-of-pocket card says there is no benefit to estimate and links to the Cardinal costs.
5	On the order, read the total off the Cash Pay card.	Cardinal cost × 1.25 per line, plus \$10 shipping on very small orders.
6	Press Generate link , then Send to patient .	The first mints the Stripe link, the second texts it. Two presses on purpose — nothing goes to a patient without a rep pressing send.
7	Wait for payment, then place the order.	Order Status flips to Paid Cash on its own when Stripe reports the payment, and the Cash Pay card says so. Check the card before you flip Order Status — nothing else will. See section 10.
8	If they pay another way, a manager releases it with a typed reason.	Cheque or over the phone. The reason is stamped into the order's notes — the only record of why goods went out against no Stripe payment.

Re-pressing Send is a chase, not a mistake

It re-texts the same link and re-stamps the date, so the date always answers “when did we last text them”.

Generate behaves the opposite way on purpose: an order that already has a link gets that link back rather than a second one, because two live links means the patient holds two and paying the older one charges the older price.

12 When something does not work

Every cash pay step reports what it did on the order row itself, in the **Cash Pay Action** column. Read that column first — it answers most of these without anyone opening a log.

What you see	What it means	What to do
Generate link or Send to patient are greyed out, with a line of text under them.	That line is the reason. Usually the feature switch has been turned off, or the order is already paid, or the quote could not be priced.	Read the line. Nothing on the board needs fixing.
You pressed Generate and nothing appears to have happened.	Look at Cash Pay Action . Empty, with a link in Cash Pay Link , means it worked. Still reading <i>Generate link</i> means the webhook never fired or was refused.	If it worked, refresh. If it is stuck on <i>Generate link</i> , see the webhook check below.
Cash Pay Action reads Link failed .	The payment service refused to mint a link and said why in its own log.	Check the coins-form-payment log on Railway for the reason, fix it, then press Generate again.
Cash Pay Action reads Text failed .	The link exists; the text did not send. Almost always a missing or unusable mobile number on the row.	Fix the number, then press Send to patient again. The link is not re-minted.
The card says it has had no answer yet after about 45 seconds.	That is not a failure. The mint may still be in flight.	Wait and refresh the page. Do not press Generate again — that risks a second link.
The patient says they paid, but the order still reads unpaid.	Payment is recorded by Stripe writing two columns: the charge id and the paid date.	Check Stripe Charge ID and Cash Pay Paid Date on the order. If they are blank, the payment did not reach us — ask Josh to check Stripe.
The card refuses to price the order at all.	One of the products has no price on the Cardinal SKU Tracker. The whole quote is refused on purpose — a total that silently leaves a product out is the one failure this cannot have.	Get the product priced on the tracker, or take it off the order.
The order changed after the link was sent, so the amount is now wrong.	A link's amount is fixed when it is minted. The card will not quietly re-price it.	Clear Cash Pay Link on the board, then press Generate link again. Replacing a link is deliberate and visible.

The trap: a dead automation that looks like a broken one

If you find an automation in monday reading “*When Cash Pay Action changes to Generate link, send a webhook*” that is **disabled with a red banner about authentication**, it is a **corpse**. Two webhooks were deleted on 22 September while the authentication was being fixed, and monday leaves the disabled entry behind with its old failure attached. **Do not press “Update automation” and do not re-enable it**. It points at the old address: at best it fails again, at worst it comes back as a third webhook on the same column and the patient is charged twice. Delete it.

How to check the two webhooks are healthy

Question	Answer
Which two are live?	641121194 — fires on <i>Generate link</i> . 641125712 — fires on <i>Send to patient</i> . Both on the New Order Board.
Which are dead?	641115241 and 641118584 . Any note, screenshot or message naming those is stale.
How do I tell if they are working?	By their run history on the board, not by the fact that they are listed — monday lists a suspended webhook exactly like a healthy one. As of 23 September: 198 successes, 0 failures since the fix.
What did the two failures on 22 September mean?	monday signs every delivery with its own credential, which was hiding ours, so the payment service refused them. Fixed the same afternoon; the webhooks were deleted and remade, which is why the ids changed.

If the shared secret is ever changed

The webhook address carries a secret. Changing it on the payment service **breaks both webhooks until they are deleted and recreated** with the new address — monday stores the address, so the old pair would be refused on every delivery. That is exactly the failure of 22 September. Change the secret and the webhooks together, never one without the other.

13 Reference

Boards

Board	Id
Profile Send Off	18406352652
Welcome Call	18410804557
New Order Board	18405457690
Cardinal SKU Tracker	18420366344
Subscription	18407459988

Cash pay columns on the New Order Board

Column	Id	Written by
Cash Pay Amount	numeric_mm7devxs	The app, before flipping the trigger
Cash Pay Action	color_mm7e3rxj	The app; cleared by the service
Cash Pay Link	text_mm7dzgzd	The payment service
Cash Pay Link Sent	date_mm7d7wxe	The text automation
Cash Pay Paid Date	date_mm7dejzt	Stripe's webhook
Stripe Charge ID	text_mm7dkma5	Stripe's webhook
Order Status	status	"Paid Cash" is label id 6

Groups referenced

Group	Id	Board
Welcome Call	group_mm1wvq8p	Welcome Call
Completed	group_mm1y57sz	Profile Send Off

The two switches

Switch	File	State
CASH_PAY_SKIPS_TO_WELCOME_CALL	src/lib/profile/cashPayIntake.ts	false
CASH_PAY_LINK_FROM_COMMAND_CENTER	src/lib/orders/config.ts	false

Each is held at false by a test. That test failing is the reminder to read the runbook before flipping it — it is not a nuisance to route around.

Where the detail lives

What	Where
The full design, and every decision behind it	CLAUDE.md §5.48, command-center-test
The skip automation: mappings, audit, UI steps	scripts/cash-pay/README.md
The payment link: the four remaining steps	scripts/cash-pay/PAYMENT_LINK.md
The pricing rule and its Debbie Hinze test	src/lib/orders/cashPayPricing.ts
The ordering gate and the manager release	src/lib/orders/cashPayGate.ts
The payment service half	coins-form-payment, backend/src/cashPay/

14 Still open

Question	Whose call
The ordering gate is off. Until the Command Center places orders itself, nothing stops an unpaid cash pay order being sent to Cardinal — a person has to read the Cash Pay card first. This is the one open item that can cost money.	Josh
A dead automation is still sitting in monday — disabled, with an authentication banner, left over from the two webhooks deleted on 22 September. It does nothing, but it invites somebody to re-enable it, which would charge a patient twice. Delete it.	Brandon
Nothing chases an unpaid link. The 15-day reminder the handoff asks for is not built — today somebody has to notice an order with a sent date and no payment. It belongs on the order board as a date-arrival automation.	Josh
The Stripe page shows one line naming the goods rather than the three products quoted. The total is identical. An itemised route is built and tested but is not what the board calls — wiring it means a second copy of the pricing rule in another repo.	Josh
23 dead mappings on the skip automation point at Medical Evaluation columns. They write nothing; worth deleting while you are in there.	Brandon
Should the doctor block appear on the Care Coordinator card for cash pay patients?	Katie